

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 34

Age: 81 years old | Sex: Female

MRN: 000-TEST-34

Admission Date: [Admission Date] | Discharge Date: [Discharge Date]

Attending Physician: Dr. M. Richardson, MD | Orthopedic Surgery

****PRIMARY DIAGNOSIS****

Severe osteoarthritis, left hip — s/p left total hip arthroplasty (THA) with cemented prosthesis

****SECONDARY DIAGNOSES****

- Vascular dementia (moderate, baseline ambulatory with supervision)
- Severe osteoporosis with history of vertebral compression fractures
- Hypertension (controlled)
- Hyperlipidemia
- Type 2 diabetes mellitus (diet-controlled)
- History of GERD

****HOSPITAL COURSE****

Patient 34 was admitted for elective left THA on [Date] due to progressive, severe osteoarthritis with intractable pain refractory to conservative management. Preoperative workup included CBC, BMP, PT/INR, and CXR—all within acceptable limits. Patient tolerated cemented THA with posterolateral approach without intraoperative complications. EBL ~300

mL. Epidural analgesia used intraoperatively; hemostasis achieved. Post-op course notable for:

- Post-op day (POD) 1: Patient alert, oriented to person only (consistent w/ baseline dementia); tolerating PO; ambulating with PT using walker, pain controlled on multimodal regimen
- POD 2-3: Mild serous drainage from surgical site; cultures obtained (pending). Continued PT/OT. Compression dressing changed per protocol.
- POD 4: Wound erythema resolved. Patient discharged to skilled nursing facility (SNF) with continued intensive therapy

****KEY INTERVENTIONS:****

- Left THA, cemented prosthesis, posterolateral approach
- Sequential compression devices (SCDs) × 3 days post-op
- Foley catheter d/c'd POD 2
- DVT prophylaxis: pharmacologic + mechanical

****DISCHARGE MEDICATIONS****

| Medication | Dose | Frequency | Duration | Notes |

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| Apixaban | 5 mg | BID | 10 days | DVT prophylaxis |

| Acetaminophen | 650 mg | TID | PRN pain | Max 3 g/day |

| Oxycodone | 5 mg | Q4-6h | PRN pain | Use cautiously; dementia risk |

| Ibuprofen | 600 mg | TID | 7 days then d/c | With food |

| Omeprazole | 20 mg | q.d. | Ongoing | Continue home dose |

| Lisinopril | 10 mg | q.d. | Ongoing | Continue home dose |

| Atorvastatin | 20 mg | q.d. | Ongoing | Continue home dose |

| Alendronate | 70 mg | Weekly (Mondays) | Ongoing | Osteoporosis; upright × 30 min |

****WEIGHT-BEARING & ACTIVITY STATUS****

- ****Left lower extremity: TTWB (toe-touch weight-bearing)**** × 6 weeks, then progress to WBAT as tolerated per PT reassessment
- ****Hip precautions (STRICT):****
 - NO hip flexion >90°
 - NO internal rotation or hip adduction
 - Use elevated toilet seat, firm chair (not low sofa)
 - Sleep supine or on non-operative side with abductor pillow between legs
 - NO bending to pick